جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : صباح سليمان بشاى يونس
المحافظة : القاهره **الحي / المدينة :** المعصره **العنوان :** زهراء حلوان
رقم البطاقة : 26303212102246 **رقم التليفون :** 01208285370 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 21-03-1963 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الاجابات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: صباح سليمان بشاى يونس
National ID: 26303212102246

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق شخصية



صباح
سليمان بشاى يونس
٤٢ ش ١ - زهراء حلوان
المعصرة - القاهره

٢٦٣٠٣٢١٢١٠٢٢٤٦

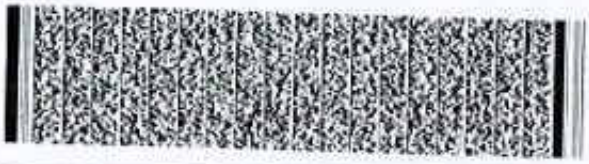
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٢٦٣٠٣٢١٢١٠٢٢٤٦ ٢٠٢٥/٠٨
حاصلة على دبلوم معهد فنى تجارى

انثى مسيحية متزوجة
عماد شفيق ندا ابراهيم
البطاقة سارية حتى ٢٠٣٢/٠٨/١٧



National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

05/12/2026

MDT FORM

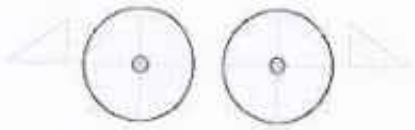
Name:	Sabah سحر حامد	**MRN:**	9706
Age:	63 y	**ID:**	28303222102240
Menopaus: Status:	POST	**Phone:**	01221447008
NRS SCORE:	PS 0	**Marital Status:**	MARRIED
BMI:	20	**Family History:**	+VE
CO Morbidity:	PREDIABETIC	**Reason Of MDT:**	NEW CASE

Clinical Data:

BX: IDC G3

ER 0/8 **PR** 0/8 **HER** -1 **KI6;** 80%

R L



Sonomammography:

ACR C, RT UO CENTRAL MASS 2.3*2.7, 3 CM FROM NIPPLE,6 MM DEEP TO SKIN.
ANT DUCTAL EXTENSION 3MM IN CALIBER.
NON-SPECIFIC LNS

Mets Workup:

CT CAP WITH CONTRAST_FREE
BONE SCAN :-FREE

TNM

History:

Recommendations:

FOR POSSIBLE CLIPPING, PORTA CATH INSERTION,
NACT THEN RT CENTRAL QUADRENTECTOMY AND

MDT Outcome & Plan

For:
1.CESM and if unifocal Clip insertion
2.BRCA testing
3.Echo
4.Portacath insertion
5.Then start NACT Keynote 522 : Taxol/Carbo x12 then AC x4 with concomitant Perjeta/Herceptin
Re-MDT after CESM for surgical decision

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE.MDT_FORM]]

GUSTAVE / ROUSSY

Patient Name	: الأستاذه / صباح سليمان بشاى	Request No.	: 13005396
Patient No.	: 26303212102246	Request Date	: 10/03/2026
Path No.	: 671/26	Verification Date	: 16/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 62 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c).
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Gross Examination:
- Three fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Absent.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL

Patient Name	: الأستاذه / صباح سليمان بشاى	Request No.	: 13005396
Patient No.	: 26303212102246	Request Date	: 10/03/2026
Path No.	: 671/26	Verification Date	: 16/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 62 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 60%.

Conclusion:
Right breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Low (score 1+).
- Ki67%: 60%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2,

Verified by :
Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE.IHC]]

GUSTAVE ROUSSY INTERNATIONAL

Patient Name	الأستاذه / صباح سليمان بشاى	Request No.	13005396
Patient No.	26303212102246	Request Date	10/03/2026
Path No.	671/26	Verification Date	16/03/2026
Sex	Female	Financial Category	جوستاف روسي - مباشره
Age	62 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient's Name:	**SABAH SOLIMAN BOSHAY BOLIS**
Date:	**2026-04-06**
Referred by:	Prof.Dr. **CT CAP IV**
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Right breast soft tissue mass measures 17x15 mm
- Few right pectoral lymph nodes measures 11x9 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

Right breast soft tissue mass for US correlation.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

GUSTAVE ROUSSY
INTERNATIONAL EGYPTE

Patient's Name:	**SABAH SOLIMAN BOSHAY BOLIS**
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Referred by:	Prof.Dr. CT CAP IV
Hospital:	**Goustave Roussey International Egypt**

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq

CT Scan
[[DOCTYPE:CT_SCAN]]

alfa
SCAN

Name: Sabah Soliman Beshay *ID: 5140974*
Date: 20/04/2026

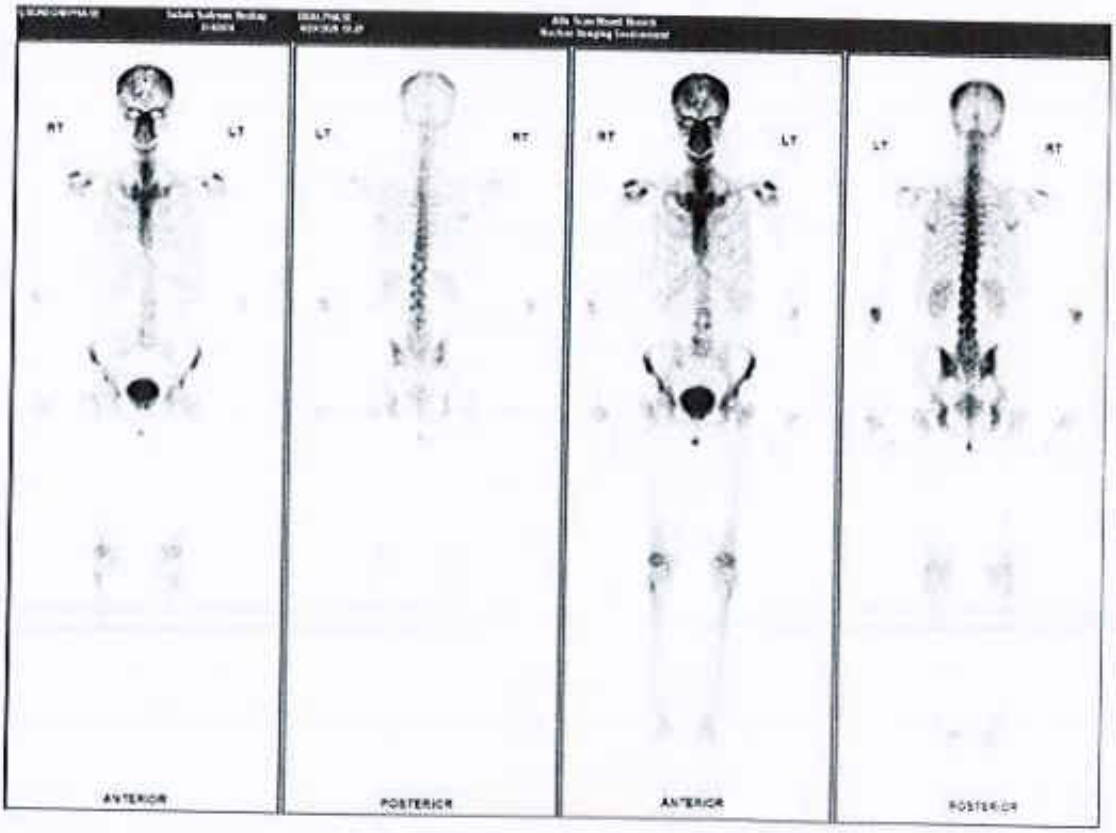
^{99m}Tc-MDP wholebody Bone Scan

Clinical data:

Indication	Right breast cancer.
Tumor markers	No.
Skeletal trauma/ surgeries	No.
Previous imaging	Not available.
Previous bone scan	No.

Technique:

Injected dose	740 MBq
Images	Delayed whole body +spot images of the skull and chest.
Complementary images	No.



alfa SCAN

Findings:

- No abnormal osseous tracer uptake detected that would account for osseous lesions.
- Arthritic changes of both shoulders are noted.

Impression:

- **No scintigraphic evidence of metastatic osseous deposits.**

Dr. Mona Saied, MD *Dr. Omnia Alaa, MSc*

ID: 5140974

تقرير موجات صوتية على القلب

	السن 63	Sabah Soliman	التاريخ 2026/6/14

Echo report

LVDd	3.5	4.2-5.8cm	FS	25	25-43%
LVSd	2.4	2.5-4cm	EF	58	>55%
IVSd	0.7	0.7-1	AO	2.8	2.6-3.0cm
LVPWd	0.8	0.7-1	LA	3.6	3-4cm

Measurements:

Normal LV dimensions with good systolic function by MM.

- Normal LV wall thickness
- SWMA: Not apparent
- RV wall dimensions, wall thickness , function, TAPSEgood
- Aortic root: normal
- LA size:normal
- RA size:Normal
- Septae: Normal

Valves:

- Normal AV morphology and flow

Normal MV and flow

- Normal TV and flow
- Normal PV and flow

Pericardium: normal

Intracardiac masses: no

Conclusion:Diastolic dysfunction

Dr Ahmed Maher Msc
Cardiology

Patient ID: 95711	Patient Name: English, Sabah Soliman Ibrahim
Age: 63 Years	2021/03/09 03:32:04 PM Report Date:

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 63-year old female with positive family history (paternal side grandmother) complaining of right mastalgia.

Technique:

Medio-lateral oblique and CC views. High resolution ultrasound

Detailed Findings:

Right breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- Outer central, middle third, irregular speculated high density mass lesion with obscured margins and related coarse heterogeneous and linear microcalcifications.
- Ultrasound demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance.
- 9 o'clock, middle third, 3 cm from nipple, 6 mm deep to the skin, irregular anti-parallel heterogeneous hypoechoic solid mass lesion with moderate internal vascularity measuring 23 x 27 mm.
- Anterior ductal extension is seen reaching the nipple through segmental prominent soft tissue echogenicity filled ducts seen reaching 3 mm in caliber.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance with no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

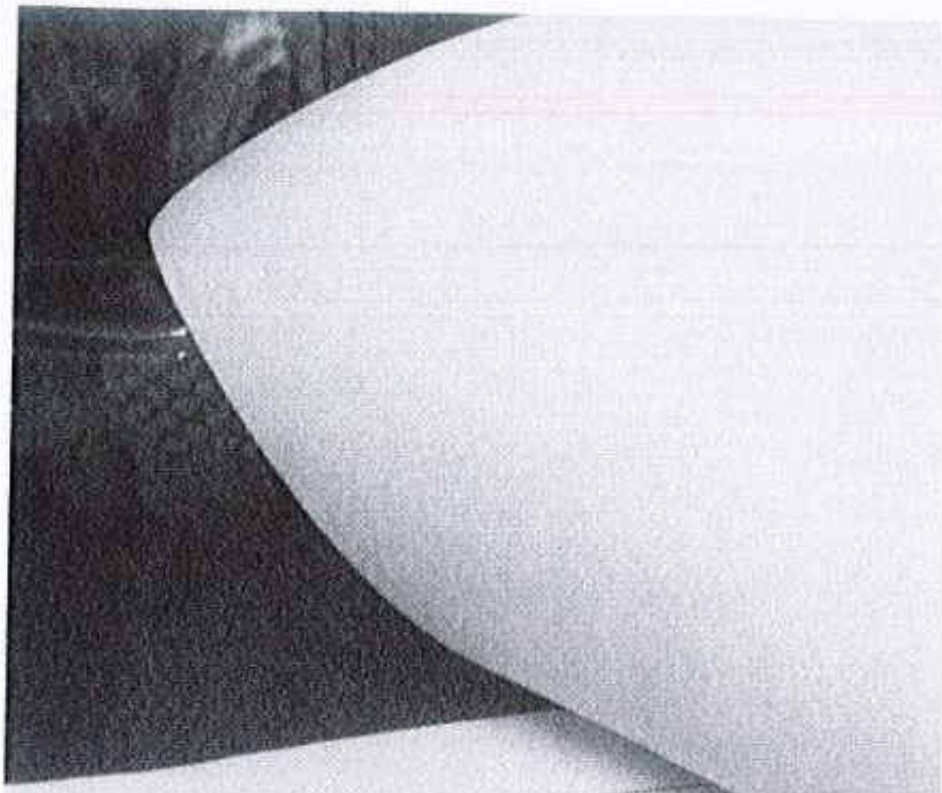
Right breast (BIRADS 4c): Outer central suspicious mass lesion with anterior ductal extension reaching the nipple. US-Guided Tru-cut biopsy was taken at same setting.

Left breast (BIRADS 1): Diffuse breast adenosis.

Recommendations:

For histopathological correlation.

Dr.Amal Hegazi, M.Sc.



GUSTAVE ROUSSY

Patient's Name:	SABAH SOLIMAN DOSHAY BOLIS
Date:	2026-04-06
Referred by:	Dr/Dr. CT CAP IV
Hospital:	Gustave Roussy International Egypt

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticula outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

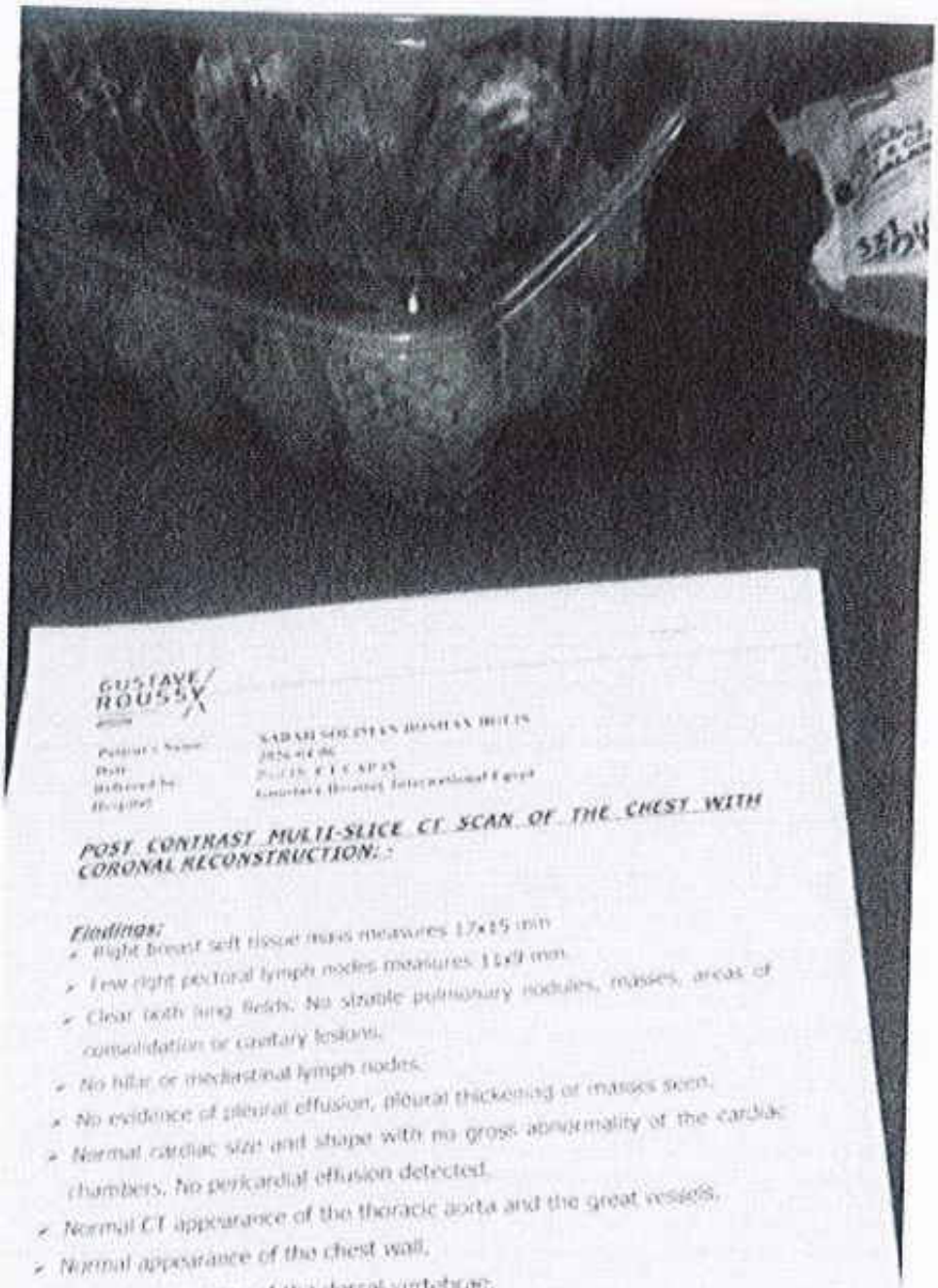
- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq

CT Scan
[[DOCTYPE:CT_SCAN]]



GUSTAVE ROUSSY

Patient's Name: SARAH SOLEIMAN BOSHRA BOUS
DOB: 28/04/86
Referred by: DR. ELIAS
Designed: Gustave Roussy International Cancer

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

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- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

Right breast soft tissue mass for US correlation.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.





GUSTAVE ROUSSY

Patient's Name: SARAH SOLIMAN SOBHAY BOLOS
Date: 2024-04-06
Referred by: DOCTOR CT SCAN
Hospital: Gustave Roussy International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Right breast soft tissue mass measures 17x15 mm.
- [illegible] right pectoral lymph nodes measures 11x9 mm.
- Clear both lung fields, no sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
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OPINION:

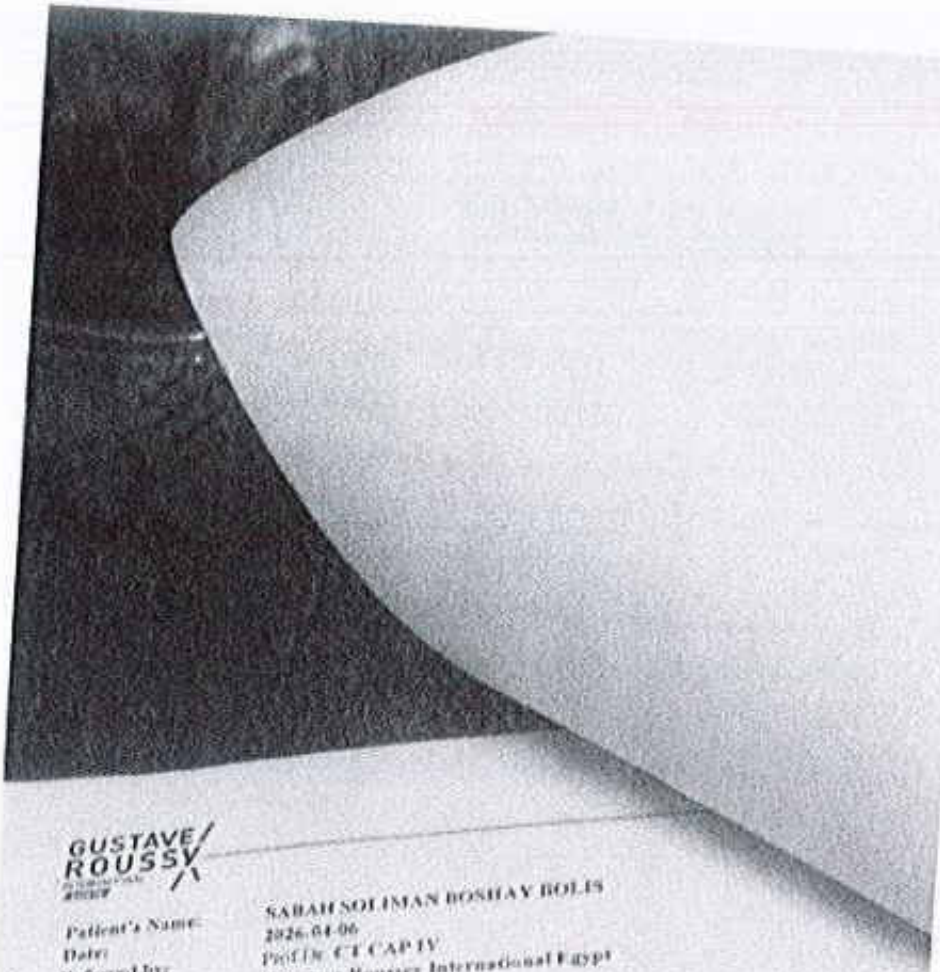
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GUSTAVE ROUSSY

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Date:	2026.04.06
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Hospital:	Gustave Roussy International Egypt

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- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq

CT Scan
[SCCTPECT_SCAN]

PATIENT NAME			Registered	08-06-2026 17:23:08	Collected	08-06-2026 17:24:19
صباح سليمان بشاى			Authenticated	07-07-2026 15:08:47	Printed	14-07-2026 10:02:46
Visit Number	Age	Gender	Referred By		Client ID	
50726508318	63 Year	Female	Prof : -		55452	

MOLECULAR BIOLOGY REPORT Report

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in *BRCA1* or *BRCA2* genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.

- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical

Rania Darwish.

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

19014
almokhtabar.com
CAP ACCREDITED
حب نفسك

PATIENT NAME			Registered	08-06-2026 17:23:08	Collected	08-06-2026 17:24:19
صباح سليمان بشاى			Authenticated	07-07-2026 15:08:47	Printed	14-07-2026 10:02:46
Visit Number	Age	Gender	Referred By		Client ID	
50726508318	63 Year	Female	Prof : -		55452	

findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.

- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

PATIENT NAME			Registered	08-06-2026 17:23:08	Collected	08-06-2026 17:24:19
صباح سليمان بشاى			Authenticated	07-07-2026 15:08:47	Printed	09-07-2026 13:37:24
Visit Number	Age	Gender	Referred By		Client ID	
50726508318	63 Year	Female	Prof : -		55452	

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in *BRCA1* or *BRCA2* genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.

- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

Rania Darwish.

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PATIENT NAME			Registered	08-06-2026 17:23:08	Collected	08-06-2026 17:24:19
صباح سليمان بشاى			Authenticated	07-07-2026 15:08:47	Printed	09-07-2026 13:37:24
Visit Number	Age	Gender	Referred By		Client ID	
50726508318	63 Year	Female	Prof : -		55452	

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.

- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

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COLLEGE of AMERICAN PATHOLOGISTS
نفسك

رئيس مجلس الادارة
ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

المختبر
al mokhtabar
moamena kamel lab
معامل مؤمنة كامل

اسم الفرع حدائق حلوان
مدير الفرع د. نسرين ابراهيم
مدير الوحده ا.د. هالة شبيه
طبيب الوحده د. ايه المرعشلي
د. دعاء خميس
د. فاتن رمضان

PATIENT NAME			Registered	17-06-2026 15:31:26	Collected	17-06-2026 15:31:45
صباح سليمان يشاى			Authenticated	18-06-2026 01:47:38	Printed	18-06-2026 21:02:39
Visit Number	Age	Gender	Referred By		Client ID	
50726508879	63 Year	Female	Prof : -		467	

HEMATOLOGY REPORT

Test Name	Result	Unit	Reference Range	Previous Result
Coagulation Profile				
Prothrombin Time				
Prothrombin Time	10.5	sec	8.2 - 13.1	
Control (MPPT):	11.1	sec		
Prothrombin Concentration	109.3	%	70 - 120	
INR	0.97		0.9 - 1.27	

PT concentration results use calibration curve Log-Log with point-to-Point (not linear) Interpolation.
Sample Type: Citrated Plasma
Method: Optical Clot-Based

INR values should only be used when evaluating patients on oral anticoagulant therapy.
Target Therapeutic Values:
INR 2.0 - 3.0 : Prophylaxis of venous thromboembolism, treatment of venous thrombosis (following heparin therapy), prevention of systemic embolism (atrial fibrillation, valvular heart disease, bioprosthetic heart valves, acute myocardial infarction).
INR 2.5 - 3.5 : Mechanical prosthetic heart valves, recurrent systemic embolism.

Hala Sheba

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ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

اسم الفرع حدائق حلوان
مدير الفرع د. نسرين ابراهيم
مدير الوحده ا.د. هالة شبيه
طبيب الوحده د. ايه المرعشلي
د. دعاء خميس
د. فاتن رمضان

PATIENT NAME			Registered	17-06-2026 15:31:26	Collected	17-06-2026 15:31:44
صباح سليمان بشاى			Authenticated	18-06-2026 00:45:50	Printed	18-06-2026 21:02:38
Visit Number	Age	Gender	Referred By		Client ID	
50726508879	63 Year	Female	Prof : -		467	

HEMATOLOGY REPORT

Test Name	Result	Unit	Reference Range
Complete Blood Picture			
Haemoglobin (EDTA Blood)	13.3	g/dL	11.5 - 15.5
Haematocrit (PCV)	43.8	%	36 - 45
RBCs Count (EDTA Blood)	4.87	Millions / cmm	4 - 5.2
MCV	89.9	fl	80 - 100
MCH	27.3	pg	27 - 33
MCHC	⇩ 30.4	g/dL	31 - 37
RDW-CV	13.7	%	11.5 - 15
Platelet Count	223	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	4.9	Thousands / cmm	4 - 11

	Percent Values		Absolute Values		
Neutrophils	55.6	%	2.70	x10^9/L	2 - 7
Lymphocytes	35.4	%	1.72	x10^9/L	1 - 4.8
Monocytes	7.6	%	0.37	x10^9/L	0.2 - 1
Eosinophils	0.8	%	0.04	x10^9/L	0.1 - 0.45
Basophils	0.6	%	0.03	x10^9/L	0 - 0.1

Comment: NORMAL BLOOD PICTURE.

Hala Sheba

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تقرير موجات صوتية على القلب

	63السن	Sabah Soliman	2026/6/14 التاريخ

Echo report

LVDd	3.5	4.2-5.8cm	FS	25	25-43%
LVSd	2.4	2.5-4cm	EF	58	>55%
IVSd	0.7	0.7-1	AO	2.8	2.6-3.0cm
LVPWd	0.8	0.7-1	LA	3.6	3-4cm

Measurements:

Normal LV dimensions with good systolic function by MM.

- Normal LV wall thickness
- SWMA: Not apparent
- RV wall dimensions, wall thickness , function, TAPSEgood
- Aortic root: normal
- LA size:normal
- RA size:Normal
- Septae: Normal

Valves:

- Normal AV morphology and flow

Normal MV and flow

- Normal TV and flow
- Normal PV and flow

Pericardium: normal

Intracardiac masses: no

Conclusion:Diastolic dysfunction

Dr Ahmed Maher Msc
Cardiology

Patient ID: 96372	Patient Name English: Sabah Seliman Beshay Yonis
Age: ٢ شهر ,٢ سنة	2026/06/17 10:43:49 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A 62 -year-old female patient with pathologically proven right breast invasive duct carcinoma grade III (-ve ER, -ve PR, -ve HER2 and Ki67= 60 %).

TECHNIQUE:CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Minimal background enhancement

ACR: C (heterogeneously dense breast parenchyma).

Right breast:

- **Low energy (Non contrast) images show:** Right breast upper outer quadrant irregular shaped indistinct hyperdense mass lesion with internal malignant pleomorphic microcalcifications.
- **Recombined (Post contrast) images show:**
  - Corresponding upper outer quadrant heterogeneously enhancing mass lesion (at 10 o'clock) of high conspicuity, with posterior linear non-mass enhancement, measuring overall 3.7x1.6x2 cm, being at about 3.5 cm from the nipple.
  - Few enhancing nodules are seen posterior to the index lesions with the farthest one being at 2.6 cm from it. By **complementary ultrasound,** corresponding indeterminate hypoechoic nodules are seen at 11 o'clock.
  - The overall process measures about 5 cm along its maximum dimension
- Normal skin / nipple / areolar complex.
- **An ultrasound guided clip insertion was done in the same setting.**

Left breast:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications. Multiple benign looking macrocalcific foci.
- **Recombined (Post contrast) images** show a retro-areolar enhancing asymmetry of low conspicuity, by **ultrasound,** corresponding 1 o'clock rather circumscribed oval hypoechoic lesion is noted measuring about 1X0.5 cm.
- Normal skin / nipple / areolar complex.

IMPRESSION:

Right breast: UOQ pathologically enhancing malignant process, as described above **(BIRADS 6).**

Left breast: UOQ (1 o'clock) probably benign lesion, for ultrasound follow up after 6 months **(BIRADS 3).**

Best regards,

Dr. Sherihan Fakhry, MD

Dr. Amira Shaheen, MD

alfa
SCAN

Name: Sabah Soliman Beshay ID: 5140974
Date: 20/04/2026

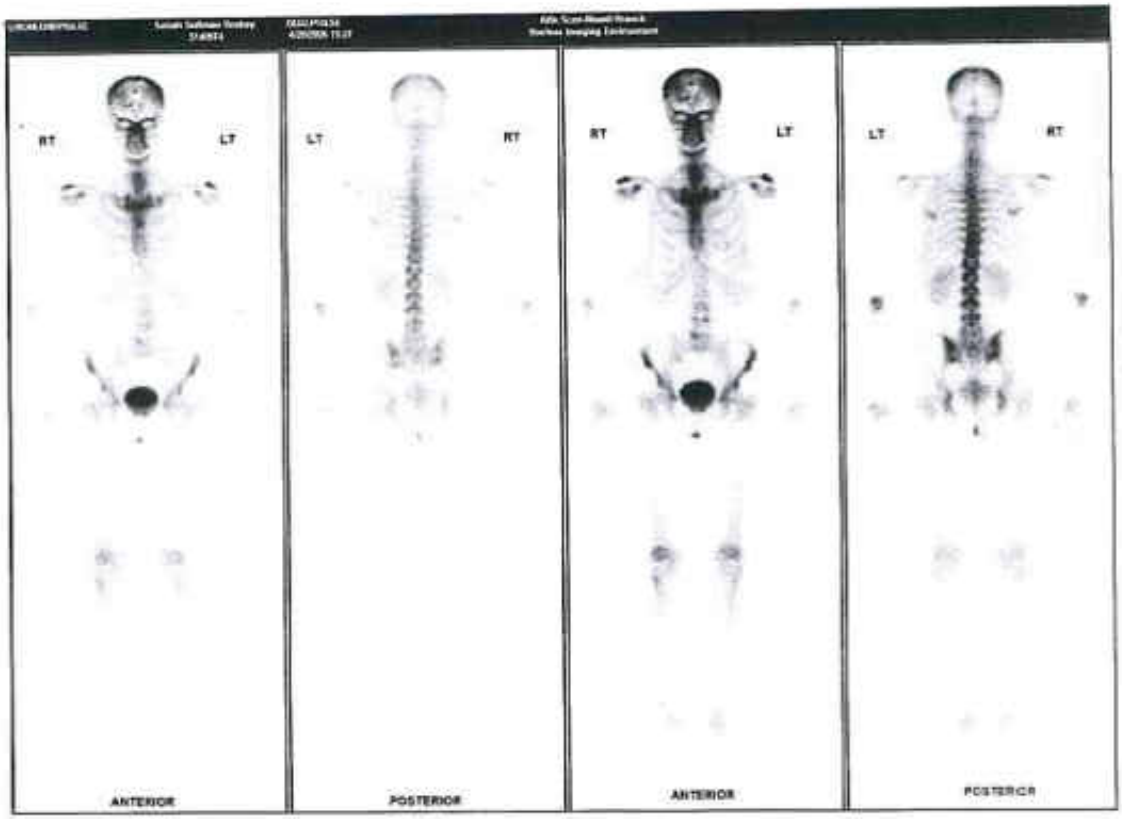
^{99m}Tc-MDP wholebody Bone Scan

Clinical data:

Indication	Right breast cancer.
Tumor markers	No.
Skeletal trauma/ surgeries	No.
Previous imaging	Not available.
Previous bone scan	No.

Technique:

Injected dose	740 MBq
Images	Delayed whole body +spot images of the skull and chest.
Complementary images	No.



alfa SCAN

Findings:

- No abnormal osseous tracer uptake detected that would account for osseous lesions.
- Arthritic changes of both shoulders are noted.

Impression:

- **No scintigraphic evidence of metastatic osseous deposits.**

Dr. Mona Saied, MD

Dr. Omnia Alaa, MSc

ID: 5140974

Patient ID: 93711	Patient Name English: Sabah Selman Beshay
Age: ٦٦ سنة ١١ شهر	2026/03/09 01:32:54 PM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 63-year old female with positive family history (paternal side grandmother) complaining of right mastalgia.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogenously dense breast parenchyma that may obscure small lesions (ACR c).
- Outer central, middle third, irregular speculated high density mass lesion with obscured margins and related coarse heterogeneous and linear microcalcifications.
- Ultrasound demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance.
- 9 o'clock, middle third, 3 cm from nipple, 6 mm deep to the skin, irregular anti-parallel heterogeneous hypoechoic solid mass lesion with moderate internal vascularity measuring 23 x 27 mm.
- Anterior ductal extension is seen reaching the nipple through segmental prominent soft tissue echogenicity filled ducts seen reaching 3 mm in caliber.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Heterogenously dense breast parenchyma that may obscure small lesions (ACR c).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance with no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Right breast (BIRADS 4c): Outer central suspicious mass lesion with anterior ductal extension reaching the nipple, US-Guided Tru-cut biopsy was taken at same setting.

Left breast (BIRADS 1): Diffuse breast adenosis.

Recommendations:

For histopathological correlation.

Dr.Amal Hegazi, M.Sc.

GUSTAVE ROUSSY
INTERNATIONAL CANCER

Patient Name	: الأستاذه / صباح سليمان بشاى	Request No.	: 13005396
Patient No.	: 26303212102246	Request Date	: 10/03/2026
Path No.	: 671/26	Verification Date	: 16/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 62 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c).
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Gross Examination:
- Three fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Absent.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

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غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / صباح سليمان بشاى	Request No.	: 13005396
Patient No.	: 26303212102246	Request Date	: 10/03/2026
Path No.	: 671/26	Verification Date	: 16/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 62 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 60%.

Conclusion:
Right breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Low (score 1+).
- Ki67%: 60%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2,

Verified by :
Dr.Duaa Mohamed

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GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	الأستاذه / صباح سليمان بشاى	Request No.	13005396
Patient No.	26303212102246	Request Date	10/03/2026
Path No.	671/26	Verification Date	16/03/2026
Sex	Female	Financial Category	جوستاف روسي - مبادره
Age	62 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

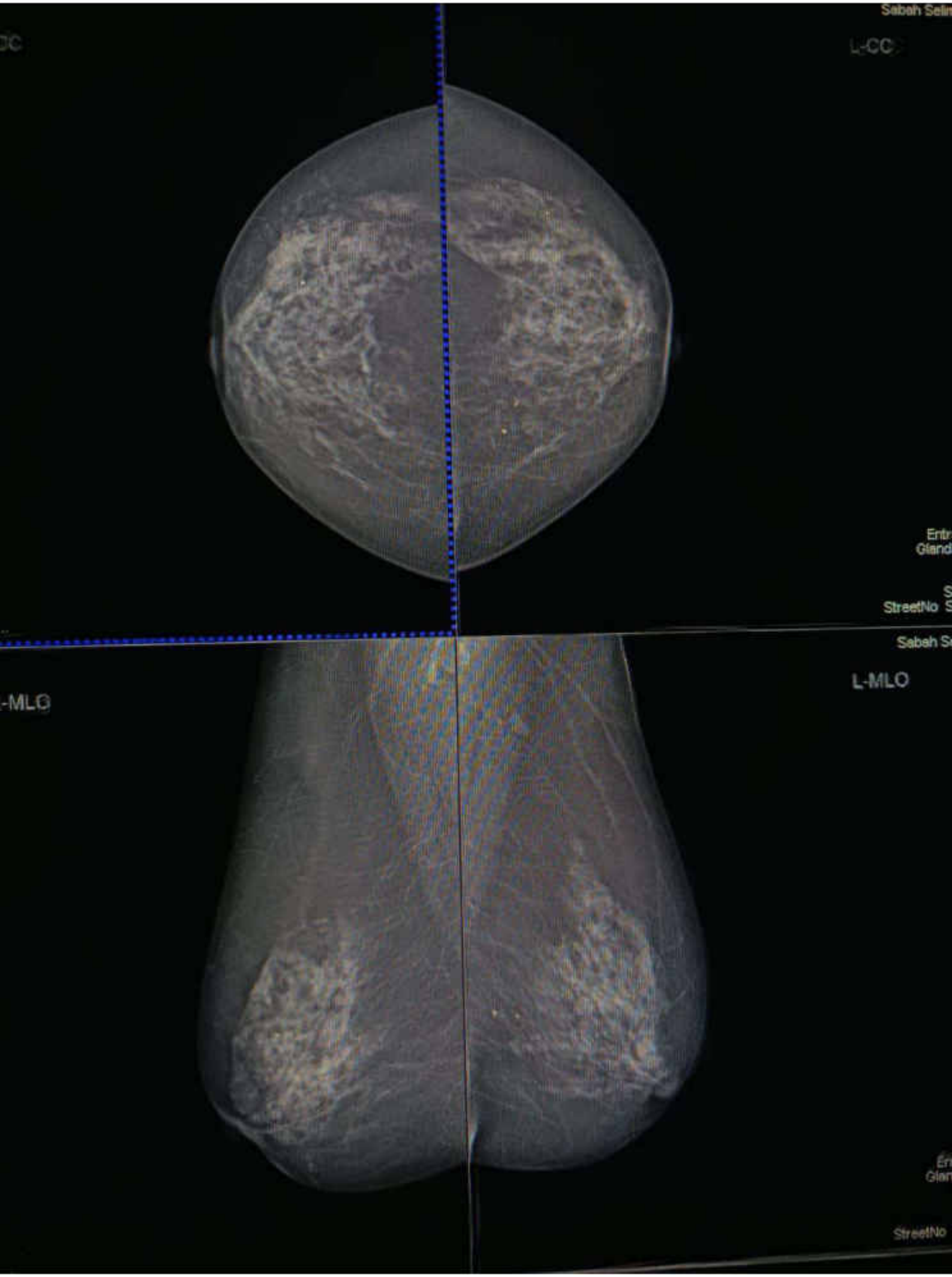
IMMUNOHISTOCHEMISTRY REPORT

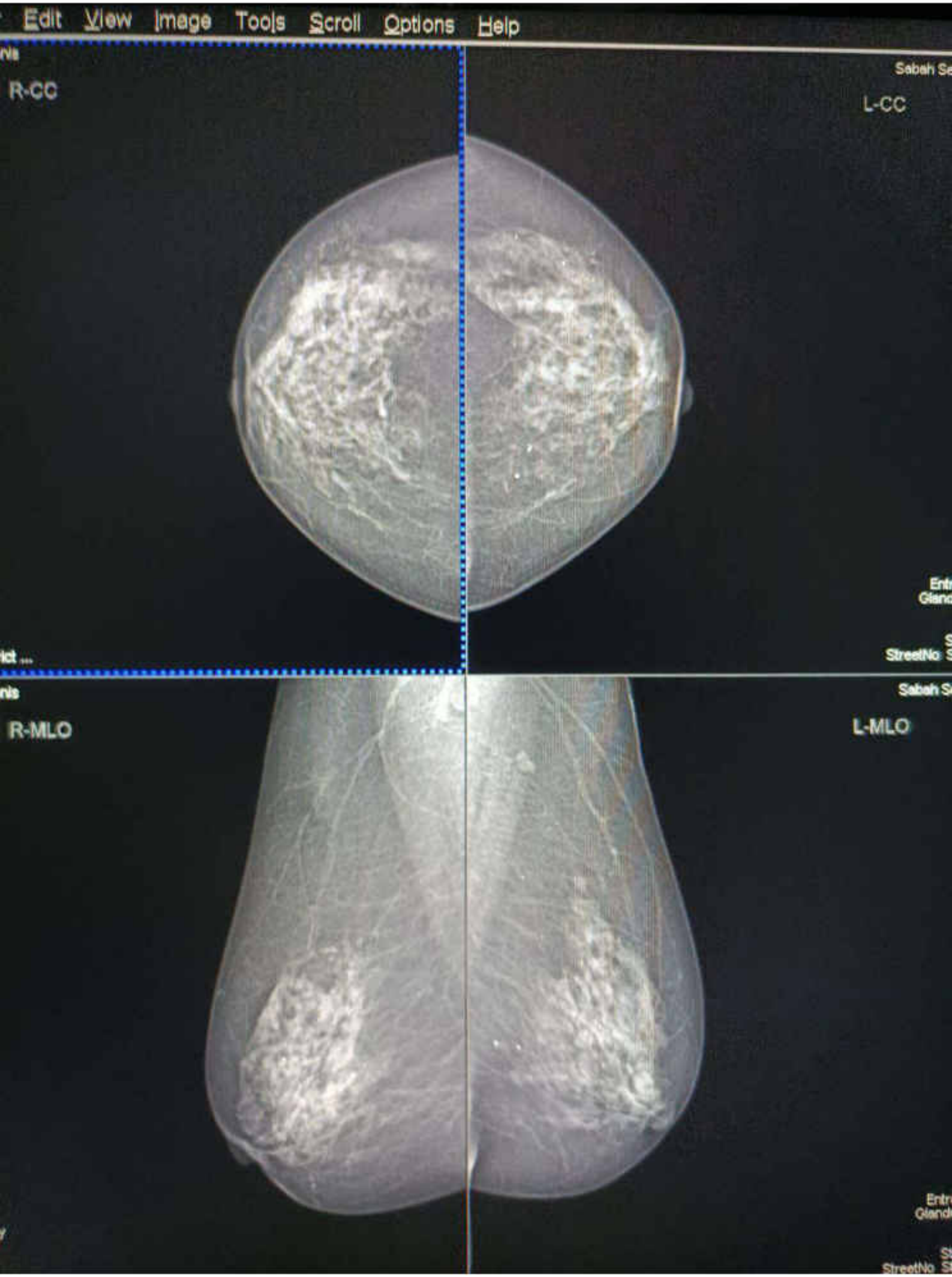
intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim.

Verified by :
Dr.Duaa Mohamed

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L-CC
L-MLO


Edit View Image Tools Scroll Options Help
R-CC
L-CC
R-MLO
L-MLO



GUSTAVE ROUSSY

Patient's Name: SABAH SOLIMAN BOSHAY BOLIS
Date: 2026-04-06
Referred by: Prof.Dr. CT CAP IV
Hospital: Gustave Roussy International Egypt

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticula outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq



GUSTAVE ROUSSY

Patient's Name: SARAH SOLIMAN BOSHAY BOLIS
Date: 2018-04-06
Referred by: Prof Dr. CT CAP IV
Hospital: Gustave Roussy International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Right breast soft tissue mass measures 17x15 mm
- Few right pectoral lymph nodes measures 11x9 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

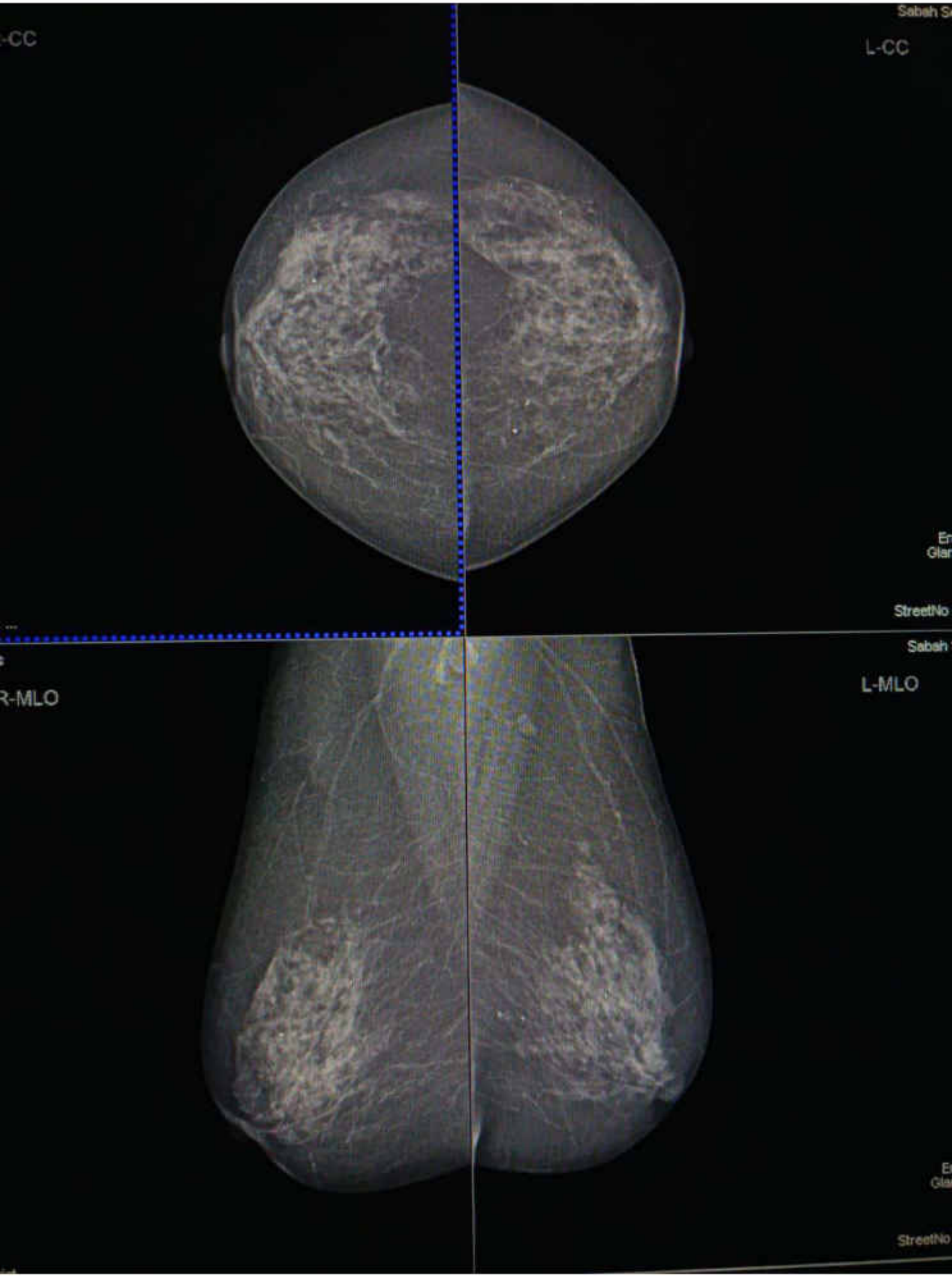
OPINION:

Right breast soft tissue mass for US correlation.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.


L-CC
L-MLO

جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة

خطاب إداري

الرقم القومي	٢٦٣٠٣٢١٢١٠٢٢٤٦
إسم المواطن	صباح سليمان بشاى يونس

تاريخ اللجنة ٢٠٢٦-١٠-٠٥	رقم الطلب	٦٨١١٤٢٦٩
	الحالة	خطاب إداري
	نص الخطاب	
جهة الإرسال جوستاف روسي انترناشيونال - مصر (EPE) *	نوافي بعدد الجرعات السابقه وعدد الجرعات المتبقيه ونوافي بالابحاث الداله علي الاستجابة وبنود صرف القرارات السابقة	
	٢٨٦٦٤ / ٣٩٢٥ / ٢١٨٢٢	

تحريراً في ٠٥-١٠-٢٠٢٦

٢٨٦٦٤ / ٣٩٢٥ / ٢١٨٢٢

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

جوستاف روسي

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـــر طبـــي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشـان تقرير طبـي للمريض: صباح سليمان شاهين

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

Breast Cancer TNBC received NACT received 10 weeks Taxol/Carbo Mhw + 4 cycles Lecytrodan for Continuation of chemo Taxol/Carbo for 2 weeks

7/10/2024

تحريرا في

الوقت: ..... التاريخ : / /

الطبيب المعالج

.....

يعتمد ،،